

CARDIOVASCULAR DECK

THE ESSENTIAL 53 CARDS

Double-Sided Study Cards · Color-Coded Categories

CLINICAL STUDY DECK SETUP

PRINTING SETTINGS

- **Paper:** Heavy cardstock, US Letter (8.5 x 11 in)
- **Scale:** 100% / Actual Size
- **Sides:** Print double-sided
- **Flip:** On the long edge
- **Color:** Full color
- **Test first:** Print pages 3-4 on plain paper to check alignment

STUDY & ASSEMBLY

- **Cut:** Follow the dashed lines, using the scissor marks as guides.
- **Recall:** Look at the colored back first and recall the details before flipping.
- **Notes:** Add your own memory tricks in the dashed box on the front.

COLOR CODING SYSTEM

Every group below has its own color, shown on both sides of each card. Use the color to group related cards and quiz yourself by class before flipping.



Beta Blockers



Lipid-Lowering Drugs



Antiarrhythmics & Digoxin



RAAS Blockers (ACE / ARB / ARNI)



Diuretics



Anticoagulants



Calcium Channel Blockers



Other BP Drugs & Vasodilators



Antiplatelets & Thrombolytics

ALIGNMENT TEST • FRONT

= 1 inch



1



2



3



4



5



6



7

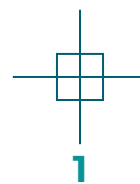
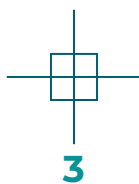


8



9

ALIGNMENT TEST • BACK



METOPROLOL

BLACK BOX WARNING

CLASS: Beta Blocker (Selective)

USE: HTN, angina, heart failure, MI

MOA: Blocks beta-1 receptors, decreasing HR and contractility

SE: Bradycardia, hypotension, fatigue, masks hypoglycemia

NOTES: Check HR/BP before giving. Do not stop abruptly.

My Notes / Clues:

© 2026 Demo Study Shop

ATENOLOL

BLACK BOX WARNING

CLASS: Beta Blocker (Selective)

USE: HTN, Angina, Acute MI

MOA: Selectively blocks beta-1 adrenergic receptors

SE: Fatigue, bradycardia, dizziness

NOTES: Hold if HR < 60 bpm. Taper when discontinuing.

My Notes / Clues:

© 2026 Demo Study Shop

PROPRANOLOL

BLACK BOX WARNING

CLASS: Beta Blocker (Non-selective)

USE: HTN, migraines, performance anxiety

MOA: Blocks Beta-1 (heart) and Beta-2 (lungs) receptors

SE: Bradycardia, bronchospasm, cold extremities

NOTES: Strictly contraindicated in asthma/COPD patients.

My Notes / Clues:

© 2026 Demo Study Shop

CARVEDILOL

CLASS: Beta/Alpha Blocker

USE: Heart failure, HTN

MOA: Non-selective beta and alpha-1 blockade (causes vasodilation)

SE: Bradycardia, orthostatic hypotension, fatigue

NOTES: Take with food to minimize orthostatic hypotension.

My Notes / Clues:

© 2026 Demo Study Shop

LABETALOL

CLASS: Beta/Alpha Blocker

USE: Severe HTN, hypertensive crisis

MOA: Non-selective beta and alpha-1 blockade

SE: Orthostatic hypotension, dizziness, scalp tingling

NOTES: Keep patient supine during IV administration.

My Notes / Clues:

© 2026 Demo Study Shop

ESMOLOL

CLASS: Beta Blocker (Selective)

USE: SVT, Atrial Fibrillation (Rapid rate control)

MOA: Ultra-short-acting beta-1 blocker

SE: Profound hypotension, diaphoresis

NOTES: Given via continuous IV infusion. Very rapid onset/offset.

My Notes / Clues:

© 2026 Demo Study Shop

LISINAPRIL

BLACK BOX WARNING

CLASS: ACE Inhibitor

USE: HTN, heart failure, post-MI

MOA: Blocks conversion of Angiotensin I to Angiotensin II

SE: Dry cough, hyperkalemia, angioedema

NOTES: Monitor BP and K⁺. Teratogenic (avoid in pregnancy).

My Notes / Clues:

© 2026 Demo Study Shop

ENALAPRIL

BLACK BOX WARNING

CLASS: ACE Inhibitor

USE: HTN, Heart Failure

MOA: Inhibits ACE, reducing Angiotensin II and Aldosterone

SE: Dry nagging cough, angioedema, hyperkalemia

NOTES: Can be given IV (Enalaprilat). Monitor renal function.

My Notes / Clues:

© 2026 Demo Study Shop

CAPTOPRIL

BLACK BOX WARNING

CLASS: ACE Inhibitor

USE: HTN, Diabetic Nephropathy

MOA: Blocks conversion of Angiotensin I to II

SE: Cough, loss of taste, neutropenia

NOTES: Shortest half-life of ACEs. Take 1hr before meals.

My Notes / Clues:

© 2026 Demo Study Shop

**BETA BLOCKER
(NON-SELECTIVE)**

© 2026 Demo Study Shop

**BETA BLOCKER
(SELECTIVE)**

© 2026 Demo Study Shop

**BETA BLOCKER
(SELECTIVE)**

© 2026 Demo Study Shop

**BETA BLOCKER
(SELECTIVE)**

© 2026 Demo Study Shop

**BETA/ALPHA
BLOCKER**

© 2026 Demo Study Shop

**BETA/ALPHA
BLOCKER**

© 2026 Demo Study Shop

ACE INHIBITOR

© 2026 Demo Study Shop

ACE INHIBITOR

© 2026 Demo Study Shop

ACE INHIBITOR

© 2026 Demo Study Shop

LOSARTAN

BLACK BOX WARNING

CLASS: Angiotensin II Receptor Blocker (ARB)

USE: HTN, diabetic nephropathy

MOA: Blocks Angiotensin II directly at the receptor site

SE: Dizziness, hyperkalemia, hypotension

NOTES: No dry cough (alternative for ACE intolerance). Teratogenic.

My Notes / Clues:

© 2026 Demo Study Shop

VALSARTAN

BLACK BOX WARNING

CLASS: Angiotensin II Receptor Blocker (ARB)

USE: HTN, Heart Failure

MOA: Blocks Angiotensin II receptors

SE: Dizziness, hyperkalemia

NOTES: Often combined with Sacubitril (Entresto) for heart failure.

My Notes / Clues:

© 2026 Demo Study Shop

IRBESARTAN

BLACK BOX WARNING

CLASS: Angiotensin II Receptor Blocker (ARB)

USE: HTN, Diabetic Nephropathy

MOA: Blocks Angiotensin II receptors

SE: Orthostatic hypotension, hyperkalemia

NOTES: Monitor BUN/Creatinine and Potassium regularly.

My Notes / Clues:

© 2026 Demo Study Shop

AMLODIPINE

CLASS: Calcium Channel Blocker (DHP)

USE: HTN, chronic stable angina

MOA: Inhibits calcium influx into vascular smooth muscle

SE: Peripheral edema, dizziness, flushing

NOTES: Educate patient to report severe swelling in legs/ankles.

My Notes / Clues:

© 2026 Demo Study Shop

NIFEDIPINE

CLASS: Calcium Channel Blocker (DHP)

USE: HTN, angina, preterm labor

MOA: Inhibits calcium influx into vascular smooth muscle

SE: Reflex tachycardia, peripheral edema, gingival hyperplasia

NOTES: Do not crush extended-release tablets.

My Notes / Clues:

© 2026 Demo Study Shop

DILTIAZEM

CLASS: Calcium Channel Blocker (Non-DHP)

USE: AFib rate control, angina, HTN

MOA: Inhibits calcium influx, slowing AV node conduction

SE: Bradycardia, peripheral edema, constipation

NOTES: Monitor HR and BP. Used IV for rapid rate control in AFib.

My Notes / Clues:

© 2026 Demo Study Shop

VERAPAMIL

CLASS: Calcium Channel Blocker (Non-DHP)

USE: Angina, SVT, HTN, migraines

MOA: Inhibits calcium influx, deeply depressing SA/AV nodes

SE: Severe constipation, bradycardia, hypotension

NOTES: Highest risk of constipation among CCBs. Increase fluids/fiber.

My Notes / Clues:

© 2026 Demo Study Shop

ATORVASTATIN

CLASS: Statin / Antilipemic

USE: Hyperlipidemia, CV risk reduction

MOA: Inhibits HMG-CoA reductase, decreasing cholesterol

SE: Myopathy, rhabdomyolysis, hepatotoxicity

NOTES: Can be taken any time of day. Avoid large amounts of grapefruit juice.

My Notes / Clues:

© 2026 Demo Study Shop

SIMVASTATIN

CLASS: Statin / Antilipemic

USE: Hyperlipidemia, CV event prevention

MOA: Inhibits HMG-CoA reductase

SE: Muscle pain, elevated LFTs, rhabdomyolysis

NOTES: Take in the evening. Report unexplained muscle aches.

My Notes / Clues:

© 2026 Demo Study Shop

**ANGIOTENSIN II
RECEPTOR BLOCKER
(ARB)**

© 2026 Demo Study Shop

**ANGIOTENSIN II
RECEPTOR BLOCKER
(ARB)**

© 2026 Demo Study Shop

**ANGIOTENSIN II
RECEPTOR BLOCKER
(ARB)**

© 2026 Demo Study Shop

**CALCIUM CHANNEL
BLOCKER (NON-DHP)**

© 2026 Demo Study Shop

**CALCIUM CHANNEL
BLOCKER (DHP)**

© 2026 Demo Study Shop

**CALCIUM CHANNEL
BLOCKER (DHP)**

© 2026 Demo Study Shop

**STATIN /
ANTILIPEMIC**

© 2026 Demo Study Shop

**STATIN /
ANTILIPEMIC**

© 2026 Demo Study Shop

**CALCIUM CHANNEL
BLOCKER (NON-DHP)**

© 2026 Demo Study Shop

ROSUVASTATIN

CLASS: Statin / Antilipemic

USE: Hyperlipidemia

MOA: Inhibits HMG-CoA reductase

SE: Myopathy, elevated LFTs

NOTES: High intensity statin. Monitor CPK if muscle pain occurs.

My Notes / Clues:

© 2026 Demo Study Shop

EZETIMIBE

CLASS: Cholesterol Absorption Inhibitor

USE: Hyperlipidemia

MOA: Inhibits absorption of cholesterol in the small intestine

SE: Hepatitis, myopathy, diarrhea

NOTES: Often combined with a statin for synergistic effect.

My Notes / Clues:

© 2026 Demo Study Shop

CHOLESTYRAMINE

CLASS: Bile Acid Sequestrant

USE: Hyperlipidemia

MOA: Binds bile acids in the GI tract, increasing cholesterol excretion

SE: Severe constipation, bloating, belching

NOTES: Interferes with absorption of many other drugs. Space dosing out.

My Notes / Clues:

© 2026 Demo Study Shop

GEMFIBROZIL

CLASS: Fibrate

USE: Hypertriglyceridemia

MOA: Decreases hepatic triglyceride secretion, increases VLDL clearance

SE: Gallstones, myopathy, hepatotoxicity

NOTES: High risk of myopathy if given concurrently with a statin.

My Notes / Clues:

© 2026 Demo Study Shop

EOLOCUMAB

CLASS: PCSK9 Inhibitor

USE: Familial hypercholesterolemia, clinical ASCVD

MOA: Monoclonal antibody that increases LDL receptor recycling

SE: Injection site reactions, nasopharyngitis

NOTES: Administered via subcutaneous injection every 2-4 weeks.

My Notes / Clues:

© 2026 Demo Study Shop

FUROSEMIDE

CLASS: Loop Diuretic

USE: Heart failure, severe edema, HTN

MOA: Inhibits Na/Cl reabsorption in ascending Loop of Henle

SE: Hypokalemia, ototoxicity, profound polyuria

NOTES: Push IV slowly (20mg/min) to avoid hearing loss. Monitor K+.

My Notes / Clues:

© 2026 Demo Study Shop

BUMETANIDE

CLASS: Loop Diuretic

USE: Heart Failure, Edema

MOA: Inhibits Na/Cl reabsorption in Loop of Henle

SE: Profound hypokalemia, ototoxicity

NOTES: 40x more potent than furosemide. Close electrolyte monitoring required.

My Notes / Clues:

© 2026 Demo Study Shop

TORSEMIDE

CLASS: Loop Diuretic

USE: Edema associated with heart failure, renal disease

MOA: Inhibits Na/Cl reabsorption in ascending Loop of Henle

SE: Hypokalemia, hypomagnesemia, dehydration

NOTES: Longer half-life and better oral bioavailability than furosemide.

My Notes / Clues:

© 2026 Demo Study Shop

HYDROCHLOROTHIAZIDE

CLASS: Thiazide Diuretic

USE: HTN, peripheral edema

MOA: Inhibits Na/Cl reabsorption in distal convoluted tubule

SE: Hypokalemia, hyponatremia, hyperuricemia, hyperglycemia

NOTES: Take in the morning. Not effective if severe renal impairment.

My Notes / Clues:

© 2026 Demo Study Shop

**BILE ACID
SEQUESTRANT**

© 2026 Demo Study Shop

**CHOLESTEROL
ABSORPTION
INHIBITOR**

© 2026 Demo Study Shop

**STATIN /
ANTILIPEMIC**

© 2026 Demo Study Shop

LOOP DIURETIC

© 2026 Demo Study Shop

PCSK9 INHIBITOR

© 2026 Demo Study Shop

FIBRATE

© 2026 Demo Study Shop

THIAZIDE DIURETIC

© 2026 Demo Study Shop

LOOP DIURETIC

© 2026 Demo Study Shop

LOOP DIURETIC

© 2026 Demo Study Shop

CHLORTHALIDONE

CLASS: Thiazide-like Diuretic

USE: HTN, edema

MOA: Inhibits sodium reabsorption in the distal tubule

SE: Hypokalemia, hyperuricemia, dizziness

NOTES: Longer duration of action than HCTZ. Preferred for HTN.

My Notes / Clues:

© 2026 Demo Study Shop

SPIRONOLACTONE

CLASS: Potassium-Sparing Diuretic

USE: Heart failure, ascites, HTN

MOA: Aldosterone antagonist in distal tubule

SE: Hyperkalemia, gynecomastia, amenorrhea

NOTES: Avoid potassium supplements and salt substitutes.

My Notes / Clues:

© 2026 Demo Study Shop

EPLERENONE

CLASS: Potassium-Sparing Diuretic

USE: Heart failure post-MI, HTN

MOA: Selective aldosterone receptor antagonist

SE: Hyperkalemia, dizziness, hypertriglyceridemia

NOTES: Less risk of gynecomastia compared to spironolactone.

My Notes / Clues:

© 2026 Demo Study Shop

ACETAZOLAMIDE

CLASS: Carbonic Anhydrase Inhibitor

USE: Glaucoma, altitude sickness, edema

MOA: Inhibits carbonic anhydrase, decreasing aqueous humor

SE: Metabolic acidosis, hypokalemia, paresthesia

NOTES: Contraindicated in patients with severe sulfa allergies.

My Notes / Clues:

© 2026 Demo Study Shop

MANNITOL

CLASS: Osmotic Diuretic

USE: Increased Intracranial Pressure (ICP), Cerebral Edema

MOA: Increases osmotic pressure of glomerular filtrate

SE: Fluid/electrolyte imbalances, pulmonary edema

NOTES: Must use a filter needle/tubing. Inspect for crystals before giving.

My Notes / Clues:

© 2026 Demo Study Shop

CLONIDINE

CLASS: Alpha-2 Agonist

USE: HTN, ADHD, Opioid withdrawal

MOA: Stimulates central alpha-2 receptors, decreasing sympathetic outflow

SE: Dry mouth, severe sedation, rebound hypertension

NOTES: Do not stop abruptly (hypertensive crisis risk).

My Notes / Clues:

© 2026 Demo Study Shop

METHYLDOPA

CLASS: Alpha-2 Agonist

USE: Hypertension in Pregnancy

MOA: Central alpha-2 agonist, reducing sympathetic tone

SE: Sedation, positive Coombs test, hemolytic anemia

NOTES: Drug of choice for pregnancy-induced hypertension.

My Notes / Clues:

© 2026 Demo Study Shop

DOXAZOSIN

CLASS: Alpha-1 Blocker

USE: HTN, BPH

MOA: Blocks peripheral alpha-1 receptors causing vasodilation

SE: First-dose orthostatic hypotension, dizziness

NOTES: Give first dose at bedtime to prevent syncope (fainting).

My Notes / Clues:

© 2026 Demo Study Shop

HYDRALAZINE

CLASS: Direct Vasodilator

USE: Severe HTN, Heart Failure

MOA: Directly relaxes arteriolar smooth muscle

SE: Reflex tachycardia, fluid retention, Lupus-like syndrome

NOTES: Often given with a beta-blocker to prevent reflex tachycardia.

My Notes / Clues:

© 2026 Demo Study Shop

**POTASSIUM-SPARING
DIURETIC**

© 2026 Demo Study Shop

**POTASSIUM-SPARING
DIURETIC**

© 2026 Demo Study Shop

**THIAZIDE-LIKE
DIURETIC**

© 2026 Demo Study Shop

ALPHA-2 AGONIST

© 2026 Demo Study Shop

OSMOTIC DIURETIC

© 2026 Demo Study Shop

**CARBONIC
ANHYDRASE
INHIBITOR**

© 2026 Demo Study Shop

**DIRECT
VASODILATOR**

© 2026 Demo Study Shop

**ALPHA-1
BLOCKER**

© 2026 Demo Study Shop

ALPHA-2 AGONIST

© 2026 Demo Study Shop

MINOXIDIL

BLACK BOX WARNING

CLASS: Direct Vasodilator

USE: Severe, refractory HTN (also alopecia)

MOA: Opens potassium channels in smooth muscle, hyperpolarizing cell

SE: Severe fluid retention, hypertrichosis (hair growth), pericardial effusion

NOTES: Extremely potent. Only used when all other HTN drugs fail.

My Notes / Clues:

© 2020 Demo Study Shop

NITROGLYCERIN

CLASS: Nitrate / Vasodilator

USE: Angina (chest pain)

MOA: Converts to nitric oxide, causing rapid venous dilation

SE: Severe headache, orthostatic hypotension, reflex tachycardia

NOTES: Keep in dark glass bottle. Call 911 if pain persists after 1 dose.

My Notes / Clues:

© 2020 Demo Study Shop

ISOSORBIDE MONONITRATE

CLASS: Nitrate / Vasodilator

USE: Angina prophylaxis

MOA: Produces nitric oxide to dilate veins and coronary arteries

SE: Headache, hypotension, dizziness

NOTES: Requires a 10-14 hour nitrate-free period daily to prevent tolerance.

My Notes / Clues:

© 2020 Demo Study Shop

AMIODARONE

BLACK BOX WARNING

CLASS: Class III Antiarrhythmic

USE: V-Fib, V-Tach, AFib

MOA: Prolongs action potential and refractory period (K⁺ block)

SE: Pulmonary toxicity, thyroid dysfunction, blue-gray skin, corneal deposits

NOTES: Extremely long half-life. Requires baseline PFTs, TFTs, and LFTs.

My Notes / Clues:

© 2020 Demo Study Shop

SOTALOL

BLACK BOX WARNING

CLASS: Class III Antiarrhythmic

USE: Ventricular arrhythmias, AFib maintenance

MOA: Non-selective beta blocker with Class III (K⁺ block) properties

SE: Bradycardia, QT prolongation, Torsades de Pointes

NOTES: Must be initiated in a hospital with continuous ECG monitoring (3 days).

My Notes / Clues:

© 2020 Demo Study Shop

FLECAINIDE

BLACK BOX WARNING

CLASS: Class IC Antiarrhythmic

USE: SVT, AFib, life-threatening ventricular arrhythmias

MOA: Potent sodium channel blockade, significantly slowing conduction

SE: Proarrhythmic (can cause lethal arrhythmias), dizziness

NOTES: Contraindicated in patients with structural heart disease or prior MI.

My Notes / Clues:

© 2020 Demo Study Shop

DIGOXIN

HIGH-ALERT MEDICATION

CLASS: Cardiac Glycoside

USE: Heart Failure, Atrial Fibrillation

MOA: Inhibits Na/K ATPase pump, increasing intracellular calcium (positive inotrope)

SE: Anorexia, N/V, visual halos (yellow/green), bradycardia

NOTES: Narrow therapeutic range (0.5-2.0 ng/mL). Hypokalemia increases toxicity risk.

My Notes / Clues:

© 2020 Demo Study Shop

SACUBITRIL/VALSARTAN

BLACK BOX WARNING

CLASS: ARNI

USE: Heart Failure with Reduced Ejection Fraction (HFrEF)

MOA: Neprilysin inhibitor (increases natriuretic peptides) + ARB

SE: Hypotension, hyperkalemia, angioedema

NOTES: Entresto. Must have a 36-hour washout period if switching from an ACE inhibitor.

My Notes / Clues:

© 2020 Demo Study Shop

WARFARIN

BLACK BOX WARNING

CLASS: Anticoagulant (Vitamin K Antagonist)

USE: DVT, PE, AFib, Valve replacement

MOA: Inhibits synthesis of Vitamin K-dependent clotting factors (II, VII, IX, X)

SE: Bleeding, bruising, tissue necrosis

NOTES: Monitor PT/INR (Target 2-3). Maintain consistent Vit K diet. Antidote: Vitamin K.

My Notes / Clues:

© 2020 Demo Study Shop

**NITRATE /
VASODILATOR**

© 2026 Demo Study Shop

**NITRATE /
VASODILATOR**

© 2026 Demo Study Shop

**DIRECT
VASODILATOR**

© 2026 Demo Study Shop

**CLASS IC
ANTIARRHYTHMIC**

© 2026 Demo Study Shop

**CLASS III
ANTIARRHYTHMIC**

© 2026 Demo Study Shop

**CLASS III
ANTIARRHYTHMIC**

© 2026 Demo Study Shop

**ANTICOAGULANT
(VITAMIN K
ANTAGONIST)**

© 2026 Demo Study Shop

ARNI

© 2026 Demo Study Shop

CARDIAC GLYCOSIDE

© 2026 Demo Study Shop

HEPARIN

HIGH-ALERT MEDICATION

CLASS: Anticoagulant (Indirect Thrombin Inhibitor)

USE: Acute DVT, PE, MI, AFib

MOA: Binds to antithrombin III, inactivating thrombin and factor Xa

SE: Bleeding, Heparin-Induced Thrombocytopenia (HIT)

NOTES: Monitor aPTT. High-alert medication. Antidote: Protamine Sulfate.

My Notes / Clues:

© 2026 Demo Study Shop

ENOXAPARIN

BLACK BOX WARNING

CLASS: Low Molecular Weight Heparin (LMWH)

USE: DVT prophylaxis, acute DVT/PE, ACS

MOA: Potentiates antithrombin III, preferentially inactivating factor Xa

SE: Bleeding, thrombocytopenia, injection site hematoma

NOTES: Lovenox. Give deep SQ in the abdomen. Do not expel the air bubble in the syringe.

My Notes / Clues:

© 2026 Demo Study Shop

APIXABAN

BLACK BOX WARNING

CLASS: DOAC / Factor Xa Inhibitor

USE: DVT/PE treatment, AFib stroke prevention

MOA: Directly and reversibly inhibits Factor Xa

SE: Bleeding, bruising, GI hemorrhage

NOTES: Eliquis. No routine blood monitoring required. Antidote: Andexanet alfa.

My Notes / Clues:

© 2026 Demo Study Shop

RIVAROXABAN

BLACK BOX WARNING

CLASS: DOAC / Factor Xa Inhibitor

USE: DVT/PE treatment, AFib

MOA: Directly inhibits Factor Xa

SE: Bleeding, bruising

NOTES: Xarelto. Take with evening meal (improves absorption). No routine INR needed.

My Notes / Clues:

© 2026 Demo Study Shop

DABIGATRAN

BLACK BOX WARNING

CLASS: Direct Thrombin Inhibitor

USE: AFib stroke prevention, DVT/PE

MOA: Direct, reversible, competitive inhibition of thrombin

SE: Bleeding, dyspepsia, gastritis

NOTES: Pradaxa. Keep in original moisture-proof container. Antidote: Idarucizumab.

My Notes / Clues:

© 2026 Demo Study Shop

CLOPIDOGREL

BLACK BOX WARNING

CLASS: Antiplatelet Agent

USE: Prevent MI/Stroke, post-stent placement

MOA: Irreversibly inhibits platelet aggregation (ADP receptor)

SE: Bleeding, bruising, TTP

NOTES: Plavix. Discontinue 5-7 days before planned surgery.

My Notes / Clues:

© 2026 Demo Study Shop

TICAGRELOR

BLACK BOX WARNING

CLASS: Antiplatelet Agent

USE: Acute Coronary Syndrome (ACS), post-stent

MOA: Reversibly inhibits the platelet P2Y12 ADP receptor

SE: Bleeding, dyspnea (shortness of breath), bradycardia

NOTES: Brilinta. Efficacy is decreased by daily aspirin doses >100mg.

My Notes / Clues:

© 2026 Demo Study Shop

ALTEPLASE (TPA)

HIGH-ALERT MEDICATION

CLASS: Thrombolytic

USE: Acute Ischemic Stroke, massive PE, STEMI

MOA: Binds to fibrin, converting plasminogen to plasmin to dissolve clots

SE: Intracranial hemorrhage, severe systemic bleeding

NOTES: Must be given within 3-4.5 hours of symptom onset for stroke. Strict exclusion criteria.

My Notes / Clues:

© 2026 Demo Study Shop

**DOAC / FACTOR XA
INHIBITOR**

© 2026 Demo Study Shop

**LOW MOLECULAR
WEIGHT HEPARIN
(LMWH)**

© 2026 Demo Study Shop

**ANTICOAGULANT
(INDIRECT THROMBIN
INHIBITOR)**

© 2026 Demo Study Shop

**ANTIPLATELET
AGENT**

© 2026 Demo Study Shop

**DIRECT THROMBIN
INHIBITOR**

© 2026 Demo Study Shop

**DOAC / FACTOR XA
INHIBITOR**

© 2026 Demo Study Shop

THROMBOLYTIC

© 2026 Demo Study Shop

**ANTIPLATELET
AGENT**

© 2026 Demo Study Shop

EDUCATIONAL USE ONLY

NOT A CLINICAL REFERENCE

This study aid is a learning and exam-review tool for nursing and pharmacy students. It must not be used to make patient-care decisions, including dosing, administration, or monitoring.

ALWAYS VERIFY

Drug information, boxed (black box) warnings, and high-alert designations change over time. Before applying anything in practice, check the current FDA-approved labeling, your facility's policies, and an up-to-date drug reference. Normal lab ranges vary between laboratories; use the ranges your program or facility provides.

RECOMMENDED REFERENCES FOR VERIFICATION

- FDA drug labels: DailyMed (dailymed.nlm.nih.gov)
- ISMP List of High-Alert Medications ([ismp.org](https://www.ismp.org))
- A current nursing drug handbook or your program's required drug reference
- Your course textbook and instructor guidance

ACCURACY

Content was reviewed for accuracy at the time of publication, but errors or omissions are possible. If you spot one, please let us know so it can be corrected in the next update.

TERMS OF USE

For personal study use by the purchaser only. You may print as many copies as you need for your own studying. Do not resell, redistribute, upload, or share this file.